CARE DIAGNOSTICS
Near Bajla Mahila College, St. Francis School Road, Castair's Town, DEOGHAR-814 112 (JHARKHAND)
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SAMPLE REPORT

NAME: TEST PATIENT DELTA **AGE/SEX: 52 / F**
REFD. BY: TEST REFERRING DOCTOR **DATE: 24/10/2026**
UHID: TEST-004 **ACCESSION: ACC-MRI-LS-002**

MRI LUMBOSACRAL SPINE

CLINICAL HISTORY:

Chronic low back pain with left leg radiculopathy

TECHNIQUE:

Multiplanar, multisequence MRI of the lumbosacral spine was performed without intravenous contrast.
Sequences acquired: Sagittal T1, T2, STIR; Axial T2.

MEASUREMENTS:

Measurement	Value
L1-L2 Canal AP	14.5 mm
L2-L3 Canal AP	13.8 mm
L3-L4 Canal AP	13.2 mm
L4-L5 Canal AP	12.0 mm
L5-S1 Canal AP	11.5 mm

FINDINGS:

Alignment

Loss of normal lumbar lordosis.

No spondylolisthesis.

Vertebral Bodies

Normal heights.

Normal signal intensity.

Bone Marrow

Normal signal intensity.

Endplates

Normal.

Intervertebral Discs

Multilevel disc desiccation from L2-L3 through L5-S1.

L1-L2

Normal disc height.

No disc bulge or herniation.

L2-L3

Mild disc desiccation.

No disc bulge or herniation.

Radiological diagnosis is not always conclusive & often vary with clinical course of the disease or response to treatment. This report is not for medico-legal purpose.

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L3-L4

Moderate disc desiccation.

No disc bulge or herniation.

L4-L5

Moderate disc desiccation.

Diffuse disc bulge causing mild thecal sac indentation.

No significant neural foraminal narrowing.

L5-S1

Severe disc desiccation.

Posterior disc protrusion measuring approximately 4 mm in AP dimension.

Moderate left neural foraminal narrowing with contact of the left L5 nerve root.

Mild right neural foraminal narrowing.

Posterior Elements

Normal.

Ligamentum Flavum

Normal thickness.

Facet Joints

Mild facet arthropathy at L4-L5 and L5-S1 bilaterally.

Normal at other levels.

Spinal Canal

Normal caliber at upper lumbar levels.

Mild narrowing at L4-L5 and L5-S1.

Neural Foramina

Patent at L1-L2, L2-L3, L3-L4 bilaterally.

Mild right narrowing at L5-S1.

Moderate left narrowing at L5-S1.

Conus Medullaris

Terminates at L1 level.

Normal signal intensity.

Cauda Equina

Normal.

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Paravertebral Soft Tissues

Unremarkable.

IMPRESSION:

1. Loss of lumbar lordosis.
2. Multilevel lumbar disc desiccation.
3. L4-L5 diffuse disc bulge with mild thecal sac indentation.
4. L5-S1 posterior disc protrusion with moderate left neural foraminal narrowing and left L5 nerve root contact.
5. Mild facet arthropathy at L4-L5 and L5-S1.

RECOMMENDATION:

Clinical correlation for left L5 radiculopathy. Consider targeted epidural steroid injection if conservative management fails.

Dr. Sugandha Priyadarshini
MD (Radiodiagnosis & Medical Imaging)

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